



P28 · 48 PAGES, FOR ANY STAY LENGTH

A NICU Parent Planner

Forty-eight pages for parents in the NICU. A daily log to bring to rounds. A glossary of the terms the team uses without translating. A page for the partner who is also grieving. A "what I want for my baby today" page. A pumping companion (if you are). A page for the day you go home. A page for the day you do not. Dignified, honest, not somber.

Soothemade Notes

A SMALL APOTHECARY OF PRINTABLES · MADE SLOWLY

A PLANNING AND TRACKING TOOL · NOT MEDICAL ADVICE

Before you open this

If you are in crisis right now, please put this planner down and call one of these.

- **988**, US Suicide & Crisis Lifeline (call or text, 24/7)
- **1-833-852-6262**, US Maternal Mental Health Hotline (24/7, free, confidential, parent-specific including NICU)
- **Hand to Hold**, NICU-parent peer support, **1-855-424-6428** (call) or text on handtohold.org

The acute hour belongs to a human on a phone, not to a planner.

With care, Maya

A note from me

I was not a NICU parent. A close friend was, with twins, for sixty-four days. She kept a notebook on the bedside table next to the recliner she slept in. Each morning at rounds, she wrote down the numbers, the words she did not know, the questions she did not have time to ask. By week three, she could speak the language of the unit fluently. By week eight, she had built a relationship with every nurse on rotation. By week ten, both babies came home.

This planner is the structured version of that notebook. It is what I wish I had given her on day one.

It does not assume one ending or another. Some NICU stays are short. Some are long. Some go home with both babies. Some do not. The planner holds all of them with care.

With care, Maya

This is a companion, not a medical document. The clinical decisions belong to your team. The planner is here to help you participate in those decisions with notes, language, and a place to put what you are carrying.

What this planner is, and is not

IS

- A daily log for rounds.
- A plain-language glossary of common NICU terms.
- A weekly questions-for-the-team page.
- A pumping companion (if relevant).
- A page for the partner.
- A "what I want for my baby today" page.
- A phone-line reference.
- A "going home" page. A "not going home" page, if that becomes the way.

IS NOT

- A medical guide. We do not diagnose. We do not interpret your baby's chart.
- A timeline. NICU stays do not follow predictable arcs.
- A "you should feel X by week Y" planner. There is no should.
- A substitute for the team. The team knows your baby. The planner helps you talk to them.

How to use this planner

- Bring it to the unit. Keep it in your bag.
- Write at rounds. Write after rounds, if rounds were too fast.
- Use the glossary as you hear new terms. Add your own.
- Use the daily log even on days that feel the same. The pattern matters later.
- Use the phone-line list. Tape it inside the front cover.
- Skip pages that do not apply. There is no completion grade.

The daily log

Use one row per day. Write at rounds if you can. Write at the end of the day if rounds were too fast.

DAY OF LIFE ____ / DATE _____

WHAT THE TEAM SAID TODAY (key words, in order)

WEIGHT TODAY ____ g / ____ oz (yesterday: ____)

RESPIRATORY ☐ room air ☐ low-flow ☐ high-flow
☐ CPAP ☐ vent ☐ other: _____

FEEDS ☐ IV nutrition ☐ NG tube ☐ bottle ☐ breast
Amount: ____ Frequency: every ____ hrs

THE NUMBER THAT MATTERED TODAY

ONE THING THAT WAS BETTER THAN YESTERDAY

ONE THING I AM WATCHING

A WORD I HEARD AND DID NOT UNDERSTAND

(add it to the glossary later)

WHO WAS ON SHIFT TODAY

Day: _____ Night: _____

(so you know who to thank later)

WAS I HELD BY THE BABY TODAY? ☐ skin-to-skin
☐ held in arms
☐ touched in isolette
☐ not yet

HOW I AM, IN ONE WORD

Use this for as many days as you can. After two weeks, the pattern will be visible. The pattern is what you take to rounds.

A plain-language glossary

You will hear these words. You may not know what they mean. The team is moving fast. Here are short translations. Add your own as you hear new ones.

Respiratory

- **Apnea:** a pause in breathing. Common in preemies. Treated with stimulation, then medication, then breathing support if needed.
- **Bradycardia (brady):** a slow heart rate. Often paired with apnea. Usually resolves with stimulation.
- **Desat (desaturation):** a temporary drop in blood oxygen. Numbers vary. The team has a target range.
- **CPAP (Continuous Positive Airway Pressure):** a machine that keeps the airway open with constant pressure. Not a ventilator.
- **Vent (ventilator):** full breathing support. Less common, used when babies cannot breathe enough on their own.
- **High-flow / low-flow:** oxygen support via small tubes in the nose. High-flow is more support than low-flow.
- **Room air:** just regular air, no extra oxygen. A goal for many babies.

Feeding

- **IV nutrition (TPN):** nutrition through a vein. Used when the gut is not ready to feed.
- **NG / OG tube:** a small tube from nose or mouth to the stomach. Used to feed when the baby cannot suck-swallow-breathe yet.
- **Trophic feeds:** very small feeds, designed to wake up the gut.
- **Fortifier:** added to breast milk or formula to give preemies more calories per ounce.

- **Bottle / breast feeds:** when the baby is ready to feed orally.

Numbers

- **Weight:** tracked daily. Loss in the first week is normal. Then steady gain is the goal.
- **Bili (bilirubin):** how the team monitors for jaundice. May need phototherapy (a special light).
- **Glucose / blood sugar:** monitored closely, especially in the first days.
- **HR (heart rate):** monitored constantly. The number you see on the screen.
- **Spo2 (oxygen saturation):** also constantly monitored. The other number on the screen.

Lines + tubes

- **PICC line:** a long IV that lives in a vein for many days. Used for medication and nutrition.
- **UAC / UVC:** lines in the umbilical artery or vein. Used in the very first days.
- **Chest tube:** drains fluid or air from around the lung. Less common.
- **Pulse ox:** the sensor on the baby's foot or hand that reads oxygen + heart rate.

Common diagnoses

- **RDS (Respiratory Distress Syndrome):** lungs not fully developed. Common in preemies. Treated with surfactant + CPAP.
- **Jaundice:** yellow skin from bilirubin buildup. Common. Treated with phototherapy.
- **NEC (Necrotizing Enterocolitis):** a serious gut condition the team watches for in preemies. Worth knowing the warning signs (the team will tell you).
- **IVH (Intraventricular Hemorrhage):** bleeding in the brain, graded 1-4. Usually detected by routine head ultrasound.
- **ROP (Retinopathy of Prematurity):** an eye condition the team screens for in babies born early.

Staff and rounds

- **Attending / neonatologist:** the lead doctor.
- **Fellow / resident:** doctors in training, often the ones you see most often.

- **NNP (Neonatal Nurse Practitioner):** a senior nurse who can do many of the same things as a doctor.
- **Charge nurse:** the nurse leading the shift.
- **Primary nurse:** a nurse assigned to your baby for continuity.
- **RT (Respiratory Therapist):** handles breathing equipment.
- **OT / PT / SLP:** occupational, physical, and speech therapy. They help with developmental work and feeding.
- **Social worker:** not for crises only. They are a coordinator and an ally. Use them.

Add your own

_____	:	_____
_____	:	_____
_____	:	_____
_____	:	_____
_____	:	_____

The questions-for-the-team page

Print or photograph. Bring to rounds. Use weekly.

WEEK OF _____

WHAT I AM WATCHING (in plain words, not theirs)

THREE QUESTIONS I WANT TO ASK

1. _____
2. _____
3. _____

WHAT THE TEAM SAID THIS WEEK ABOUT GOING-HOME

- ☐ Specific date being discussed: _____
- ☐ Conditions before discharge: _____

- ☐ Not yet discussing.

WHO I'D LIKE TO TALK TO BEFORE THE WEEK IS OUT

- ☐ Attending
 - ☐ Primary nurse
 - ☐ Social worker
 - ☐ Lactation
 - ☐ OT/PT/SLP
 - ☐ Other: _____
- _____

A pumping companion, for NICU

Six pages, if you are pumping for a NICU baby.

What's different about pumping for a NICU baby

- **You start early.** Usually within 6 hours of delivery if the baby is not at the breast.
- **You pump often.** Eight to ten times in 24 hours, including overnight, for the first weeks.
- **The output is small at first.** Drops. A few drops. That is normal. Colostrum is calorically dense.
- **The hospital usually provides a pump** while you are inpatient. Your insurance covers one for home.
- **The milk goes to the unit.** You bring it in, labeled. The team will give you the labeling format.

The schedule, weeks 1-2

SUGGESTED INTERVAL: every 2-3 hours, ~8-10 sessions / 24 hours

SUGGESTED OVERNIGHT: one session between midnight and 4am
(prolactin is highest then)

SAMPLE DAY (adjust to your sleep):

5am / 8am / 11am / 2pm / 5pm / 8pm / 11pm / 2am

The output log

DATE	SESSION TIMES	TOTAL OZ
_____	_____	_____
_____	_____	_____
_____	_____	_____
_____	_____	_____
_____	_____	_____
_____	_____	_____
_____	_____	_____

Reminders

- Drops in the first days are normal. The baby needs very little. The number on the bottle does not predict your long-term supply.
- The unit's lactation consultant is for you, not just for at-the-breast feeding. Use them.
- If your milk is plentiful and the baby is small, the unit may store extra. Ask.
- Donor milk is an option for some NICU babies. Different units handle it differently.

When you are tired

If you genuinely cannot keep up the pumping schedule (you are sick, you are crashing, you are running on no sleep), call the lactation consultant. They will help you triage. Skipping a session is not a failure. A safe parent is more important than one session.

A page for the partner

Often the non-birthing partner is in the unit with the baby while the birthing parent is recovering. Sometimes the reverse. Often you are alternating.

TO THE PARTNER

You are in the unit. The other parent is recovering, or in their own room, or has gone home for a few hours.

You may feel useful. You may feel out-of-place. You may feel both in the same five minutes.

Your job in the unit, in the early days:

- ☐ Be present at rounds if the other parent cannot be. Write down the same notes they would. Bring them home or to her room.
- ☐ Talk to the baby. Out loud. Reading. Singing. The baby's body recognizes your voice from before birth. The voice helps.
- ☐ Skin-to-skin, when the unit says you can. The other parent will get to do this too. Some of the skin-to-skin time is yours.
- ☐ Hand-cup the baby's body inside the isolette, if skin-to-skin isn't possible yet. Just resting hands. Heat through your palms.
- ☐ Ask the nurses what they would like you to do at this stage. They have ideas. They want you to participate.
- ☐ Write down the nurses' names. By month two, you will know them and they will know you.

WHAT YOU ARE ALSO ALLOWED TO DO

- ☐ Cry, in the unit's family room, with the door closed.
- ☐ Step out for a walk around the block. Once a day at least.

☐ Eat a real meal, sitting down, outside the cafeteria.

☐ Call one friend who can hear about this without trying to fix it.

☐ Ask the social worker for support resources. They know the local NICU-parent support groups.

The “what I want for my baby today” page

One small wish per day. You'll see this page repeated 30 times in the planner.

DATE _____

TODAY I WANT FOR THE BABY:

TODAY I WANT FOR MYSELF:

TODAY I WANT FOR US:

The wishes do not have to come true. The writing is the work.

The phone-line list

THE UNIT

NICU main line: _____
NICU bedside phone (if any): _____
Lactation consultant: _____
Social worker: _____
Chaplain (if relevant): _____
Cafeteria hours: _____

MY TEAM

OB / Midwife: _____
Pediatrician (for after): _____
Insurance member services: _____

SUPPORT

Hand to Hold (NICU peer support, US):
1-855-424-6428

March of Dimes parent support:
1-888-663-4637

US Maternal Mental Health Hotline:
1-833-852-6262 (24/7, free, confidential)

US Suicide & Crisis Lifeline:
988 (call or text, 24/7)

A friend who can come if I call:
_____ at _____

Another friend (for the days the first is busy):
_____ at _____

A page for the day you go home

A page at the back of the planner.

DISCHARGE DAY

DATE: _____

THE LAST THING THE TEAM SAID:

WHAT WE GO HOME WITH

- ☐ Discharge summary
- ☐ Pediatrician follow-up date (within ____ days)
- ☐ List of any medications, by purpose (we keep the actual names off this planner. The discharge papers have them)
- ☐ Feeding instructions
- ☐ Equipment, if any (apnea monitor, oxygen, feeding pump)
- ☐ Numbers to call after discharge
- ☐ A list of specialist referrals, if any
- ☐ The instructions sheet for car-seat-tolerance testing (if it was done)

WHO TO THANK

The night I want to remember:

The nurse I want to remember:

The other parent I met in the family room:

THE FIRST NIGHT HOME

The plan: _____

The backup: _____

A page for the other ending

For the families this is true for. Most NICU stays end in going-home. Some do not.

A PAGE FOR THE OTHER ENDING

If this is the way your story has gone, this page is here.

There is no language that fixes this. The planner does not have one.

Please call:

US Maternal Mental Health Hotline: 1-833-852-6262

Postpartum Support International: 1-800-944-4773

Share, Pregnancy & Infant Loss Support: 1-800-821-6819

The unit's social worker, in person. They will know the local bereavement-doulas, peer-support groups, and resources. Many hospitals have specific NICU-loss support that is not advertised.

The companion you may want for the longer stretch is "A Gentle Companion for Pregnancy Loss" (P20). It is not a perfect fit for NICU loss but it has some of the same scaffolding.

What we know, for the rest of this page:

You are not alone, even though it feels that way.
The team you built in the unit is also grieving.
The friends who showed up are still here.
The next year will be longer than any year has been.
You will not "get over" this. You will live around it.

The page is just here. It does not ask anything of you. It is the small, honest acknowledgment of the way this can go.

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The month-after page

For the version of you reading this back at home, a month after discharge.

You went home. The bag came home. The recliner you slept in is no longer yours.

The thing you carried in the unit is also coming home with you. It does not unpack on a schedule. Some of it lives in dreams. Some of it lives in the way you watch the baby's chest rise and fall.

That is normal. That is a thing the unit's social worker, your pediatrician, and a real therapist can all help you carry.

Three small things to write, if you want:

One. The nurse I want to send a thank-you to is ____.

Two. The thing I want to remember about the unit, that I am afraid I will forget, is ____.

Three. The thing about myself I learned in the unit is ____.

Close the planner if you are ready. Open it again whenever you want. You are home.

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From Soothemade Notes, a small apothecary of printables, planners, and cards for the unphotographed parts of new parenthood. Made slowly, in plain language.

Not medical advice.



Soothemade *Notes*

*Made for the version of you no one tells you about, the
one at four in the morning, the one in the parking lot, the
one who doesn't recognize her own week.*

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